

--SUMMARY--

Decision No. 10/18

01-Nov-2018

R.Nairn

- Second Injury and Enhancement Fund {SIEF} (preexisting condition)

No Summary Available

13 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w0119n]

Style of Cause:

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 10/18

BEFORE:

R. Nairn: Vice-Chair

HEARING:

September 7, 2018, at Toronto
Written

DATE OF DECISION:

November 1, 2018

NEUTRAL CITATION:

2018 ONWSIAT 3431

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) decision dated August 19, 2016

APPEARANCES:

For the worker:

M. Farago, Lawyer

For the employer:

S. Lam, Paralegal

Interpreter:

N/A

**Workplace Safety and Insurance
Appeals Tribunal**

505 University Avenue 7th Floor
Toronto ON M5G 2P2

**Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail**

505, avenue University, 7^e étage
Toronto ON M5G 2P2

REASONS

(i) Introduction

[1] At the time of the accident under consideration here, the worker (born in 1983) was employed as a carpenter in the accident employer's construction business.

[2] On March 25, 2015, (about two weeks after starting with the employer) the worker was injured when he slipped on some ice and fell at work. In his Report of Injury/Disease (Form 6), the worker described the mechanics of the accident as follows:

Slipped on the ice. My feet came out from under me. Hurt shoulder blades & ribs on ice. Hurt sternum on impact. Hurt upper back due to D ring on harness. Hurt lower back due to tool belt.

[3] In its Report of Injury/Disease (Form 7) the employer described the mechanics of the accident as "[The worker] slipped and fell, landing on his back. He was wearing a body harness with the 'D' ring located in the upper middle portion of his back".

[4] The worker sought medical attention at a local hospital and the Health Professional's Report (Form 8) of March 26, 2015 completed by Dr. J. Zrobacu provided a diagnosis of "MSK back injury post-fall".

[5] As noted in Memo No. 1 of April 10, 2015, the WSIB (the "Board") recognized the worker's back injury as compensable. The Eligibility Adjudicator allowed the claim for an upper back strain/sprain, but granted healthcare benefits only, since the worker was provided with modified duties at no wage loss.

[6] Information on file suggests that the worker stopped working on about April 13, 2015, and in a decision dated April 21, 2015, a Board Case Manager determined that the worker was entitled to Loss of Earnings ("LOE") benefits from April 13, 2015. In reaching that conclusion, the Case Manager indicated in part:

Details of the case:

[The worker] injured his back at work on March 25, 2015 when he slipped and fell. He was seen at Emergency on March 26 and diagnosed with a MSK back injury following a fall. The physician provided restrictions, referred for treatment and prescribed medication.

Effective March 26, [the worker] was provided with modified work at the work site, at reduce wages: paper work, document review, document processing, 2-3 hours per day, he was paid for 8 hours. On his next rotation out, [the worker] returned to [T] where he started treatment on April 10.

[The worker] did not return to work on April 13, the start of his next rotation.

(...)

Decision

In order to access treatment [the worker] was required to travel 2.5 hrs to [M], the closest centre where treatment could be provided. The return trip would have taken at least 5 hours and more if the worker had to stop to stretch and rest. The requirement to travel would negate any health benefits provided by the treatment. Therefore the modified duties are considered not suitable and LOE benefits are allowed from April 13, 2015.

- [7] On April 21, 2015 the worker had a CT scan of his thoracic and lumbosacral spine performed. The result were interpreted to reveal:

THORACIC SPINE:

There is a mild concave compression deformity of the superior endplate of T3. There is some associated sclerosis in the subcortical bone adjacent to the superior endplate of T3, suggesting this is remote in nature. Clinical correlation is required.

There is some mild localized concavity of the right anterolateral aspect of the superior endplate of T 9, probably representing a Schmorl's, (i .e. cartilaginous) node here.

Small anterior osteophytes are noted at several levels in the mid and lower thoracic spine.

No other bony or joint abnormalities can be identified. Specifically, no compression fracture of the T10 vertebral body can be identified.

LUMBOSACRAL SPINE:

The 5th lumbar segment is transitional in nature. articulating with the sacrum on the right via a pseudo arthrosis between a large right transverse process and the sacrum.

No other bony or joint abnormalities can be identified. Specifically, no fractures can be identified.

- [8] In May 2015, the worker was referred to the Board's Back & Neck Specialty Clinic where he was assessed by a team including Dr. R. Rampersaud, an orthopaedic spine surgeon. In the May 25, 2015 report which followed that assessment, Dr. Rampersaud noted that the worker was "a 32 year old Journeyman Carpenter who sustained a T3 compression fracture as a result of a slip and fall at work on March 25, 2015". With respect to a diagnosis, Dr. Rampersaud also noted:

DIAGNOSIS

Mid-thoracic contusion with significant myofascial soft tissue pain as well as mechanical pain with an associated T3 compression fracture. This is causing significant myofascial dysfunction in the neck, mid-thoracic and periscapular region on the right side as well as significant psychosocial and psychological factors. The main issue that this gentleman expressed to us today is that he feels his trip to Toronto was ill advised by his chiropractor and thus, as he was told he would be getting new assessments and investigations in Toronto that were not available to him in [his home city], that the aggravation of the trip due to the travel and increase of his pain would be harmful. He certainly is quite stressed out about this. We have discussed it in multiple iterations throughout the interview that we do try to offer a more comprehensive assessment and we deal with recalcitrant cases. If the treatment he was getting locally was in fact working, then there would not have been a need to have him come and be assessed. After a variety of different negative interactions, he states that he wants his pain justified. I tried to further clarify what justification is required. From the nature of his injury as well as a concurrent compression fracture and a soft tissue injury, even without the concurrent fracture, his pain is certainly related to his injury and we tried to spend time explaining that the amplification of his pain and the significant other stressors with his employer are dramatically adding to the degree and persistence of his pain. He states that his wife is a nurse and works at a chronic pain clinic and he already knows all this and again, felt that he was not hearing anything from our assessment that was of any use to him and was quite angry that he was sent here.

Be that as it may, we tried to explain to this gentleman that he needs comprehensive multidisciplinary care and with no offense to his chiropractor who seems to be giving him appropriate treatment, he needs care above and beyond this. Certainly, if it can be delivered locally, then by all means it should be and our goal is to do an independent

assessment of him to provide medical guidance. He states he is seeing a local orthopaedic surgeon. We tried to explain that unless they are dedicated spine surgeons with the experience of delivering care in an academic evidence-based setting and experience with multidisciplinary team approach to complex cases as his, then his time here is not wasted; however, that is obviously a matter of opinion and he is certainly entitled to his.

RECOMMENDATIONS AND PLAN

Based on our assessment findings as described above, we recommend the following for [the worker]:

Investigations

From our perspective, he does not require further investigation as the cause of his pain is clear; the size of his persistent and complex course is also clear with all the blue flags and psychosocial stressors.

Consultations

We feel this gentleman requires assessment and treatment with a Pain psychologist or counsellor. He is not open to attending any treatment in Toronto and as such, we recommend that he seek out these services in his community if he is agreeable to this in the future.

Surgery

He is not a surgical candidate.

Rehabilitation

We feel his current rehabilitation should be augmented with use of acupuncture which does not guarantee better pain control, but in some, can have dramatic impact on muscle tone and spasm, and as such, a limited course or trial of acupuncture to see if it can facilitate more progression of his exercises and mobility would be of significant benefit for this gentleman. In addition, he should be doing periscapular strengthening exercises to try to initiate some scapular retraction and/or latissimus dorsi retraction to offload his currently painful periscapular and thoracic muscles (i.e. rhomboids, levator scapula).

Medical Restrictions

No new recommendations in this regard.

Return to Work Recommendations

[The worker] would be fit for sedentary duties at this time, with limited walking and standing. Lifting and carrying tasks should be avoided at this time.

These recommendations are temporary and can be increased by his treating therapist in the community depending on the client's progress and response to the recommended treatment program.

PROGNOSIS

This gentleman has had some recovery to date and may continue to improve with the recommendations; however, his main barriers at this time seem to be the interpretation of what his current issues are. Although he states he has heard all this before and is aware of the issues that we brought up around pain and pain dysfunction associated with an injury, he is not responding in a way that reflects an individual that does truly understand or accepts the nature of his current problem.

(...)

[9]

In a decision dated July 14, 2015 a Case Manager dealt with the issue of the worker's ongoing entitlement to LOE benefits and concluded that his entitlement to those benefits would

cease as of July 31, 2015, at which point it was expected that he would be able to return to his pre-injury job. In reaching that conclusion, the Case Manager indicated in part:

Details of the case

You slipped and fell at work on March 25, 2014 and sustained a fracture at T3. You have been off work since April 13, 2015.

In a decision dated April 21, 2015 I allowed Loss of Earnings (LOE) benefits from April 13 when I determined the modified work was not suitable due to the travel time required to access treatment.

You saw Dr. Rampersaad at the WSIB Neck and Back specialty Clinic on May 25, 2015 and Dr. Hoffman on May 27, 2015. Both specialists confirmed the diagnosis of T3 compression fracture. Dr. Hoffman anticipated a recovery 3 months from the date of injury, so another month or month and a half (from March 27) for the pain to start easing off and probably another half month to get back into working physical condition.

Dr. Brimmell provided chiropractic care for 8 weeks, the discharge report dated June 9, 2015 indicated that you could return to modified work on June 25, 2015 with limitations against lifting, kneeling, bending/twisting, climbing stairs and ladders and the use of the upper extremities. Dr. Brimmell recommended you start with modified hours.

[The] Return to Work Specialist (RTWS), contacted the employer on June 29, 2015. They confirmed they had no modified work in [your home city] therefore I allowed LOE benefits until July 31, 2015, when you are expected to be fit to return to your pre-injury job.

[10] In July 2015, the employer asked that the Board consider granting it entitlement to Second Injury and Enhancement Fund (“SIEF”) relief from the costs of this worker’s claim. The matter was considered by a Case Manager in Memo No. 51 of July 28, 2015. With respect to the severity of the accident, the Case Manager was of the view that it ought to have been characterized as “moderate” and likely to cause a disabling injury, noting “the sudden unexpected/uncontrolled slip and fall (added peril)”. The Case Manager also however, decided that SIEF relief was not appropriate finding that there was “no evidence of a pre-existing condition that has contributed to the injury or delayed recovery”. The Case Manager then issued a decision dated July 28, 2015 confirming the denial of entitlement to SIEF relief.

[11] The employer did not agree with the conclusions of the Case Manager and the matter was eventually referred to an Appeals Resolution Officer (“ARO”). In a decision dated August 19, 2016 the ARO denied the employer’s appeal. With respect to the issue of the accident severity, the ARO indicated in part:

I considered the components of the accident history of March 25, 2015. I find that the accident is moderate in severity based on the worker's body position, mechanics, and environment at the time that he slipped and fell on ice at work on March 25, 2015. In support of this finding I relied upon the worker's evidence. In the Worker's Report of Injury/Disease (Form 6) dated April 14, 2015 the worker documented that he slipped on ice, his feet came out from under him and that he fell backward and landed onto his back. In the course of the fall the worker described that he was wearing a body harness with the D-ring located in the upper middle region of his back.

The worker's evidence is consistent with the employer's evidence.

(...)

On this basis, I am satisfied that the mechanism of injury, as described, is expected to cause a disabling injury.

[12] With respect to the medical significance of any pre-existing condition, the ARO concluded:

(...)

I find it instructive that both Dr. Rampersaud and Dr. Hoffman concluded that this worker sustained a significant work-related injury that included a T3 compression fracture that has caused significant myofascial dysfunction in the neck, mid-thoracic and periscapular regions. Despite the worker's degree of anxiety arising from his work-related injuries, there is little medical evidence to suggest that a pre-existing condition rendered this worker more vulnerable to the development of a disability of greater severity than a normal person. This is not the case. It is significant that Dr. Hoffman did not express a medical opinion to suggest that this worker required psychological treatment or intervention due to a non-organic condition.

I considered that the worker was injured due to the specific slip and fall accident that occurred at work on March 25, 2015. There is no medical evidence to suggest that the worker's upper back, middle back, lower back and T3 compression fracture injuries were not compatible with the accident history. In support of this finding I relied upon the CM's decision letters dated April 21, 2015 and July 14, 2015 that allowed initial entitlement as it was concluded that the worker's upper back, middle back, lower back and T3 compression fracture injuries were compatible with the mechanism of injury.

I find on a balance of probabilities that the recovery and claims period was not enhanced or prolonged due to a pre-existing condition.

I am persuaded that the worker had no pre-existing condition of any medical significance.

(ii) Issue on appeal

[13] The issue to be determined in this case is whether the employer is entitled to SIEF relief with respect to the costs of this worker's claim.

(iii) Submissions of the employer's representative

[14] The employer agreed to have this appeal considered by means of written submissions.

[15] Mr. Lam provided submissions dated July 31, 2018 which have been reviewed and are included in Post-Hearing Addendum No. 2. Mr. Lam requested that the employer be granted 90% SIEF relief. In those submissions, Mr. Lam indicated in part:

The ARO Decision of August 19, 2016 had erred in their failure to recognize the pre-existing psychological conditions.

[The worker's] back injuries and part of the problems was landing on his back while wearing a back safety harness & tool belt it's reasonable to assume a fall on the Harness D-Ring as the source of the physical problems. Though, this is not the only reason, the ARO ignored the unrelated medical problems that led to his eventual decision to abandon his job on July 31", 2015.

Worker Medical History

ARO denied any pre-existing conditions, the medical indicated other problems that contributed to his lack of motivation in RTW.

- Case Record 1, Memo-July 28, 2015, Page 59. "The clinic identified psychosocial stressors and blue flags, worker declined psychological counselling."

- Case Record 1, Memo-June 17, 2015, Page 79. "CM confirmed with worker he declined psych counselling to deal with Myofascial pain/anxiety issues...wife is a Nurse , she is a PHO student with a specialty in Chronic Pain...she and her colleagues have informed him he does not meet the criteria for chronic pain...the clinic also commented on his usage of Opioid. .."
- Case Record 1, Memo-June 16, 2015, Page 80. "Worker saw Dr. Hoffman on May 27, 2015. He documents a history of anxiety."
- Case Record 1, Dr. Hoffman-May 27, 2015, Page 166. "The patient does have a history of anxiety. He tells me that about 7 years ago, he sustained an injury to his upper back when he was hit by a hammer near the base of his neck."
- Case Record 1, Altum-May 25, 2015, Page 171 & 175. "He also reports that a sledgehammer hit his back approx. 8 years ago, causing soft tissue damage - significant psychosocial and psychological factor".

(...)

Conclusions

[The employer] might have employed and placed the worker outdoors, possibly exposing him to conditions that led to the slip and fall on ice. There is contradictory evidence the recovery was delayed supporting pre-existing conditions that has hampered his life. We ask the Tribunal to review the medical evidence that supports our conclusions pre-existing conditions physically and psychologically exists.

(iv) Submissions of the worker's representative

[16]

Mr. Farago provided submissions dated August 29, 2018 which have been reviewed and are included in Post-Hearing Addendum No. 2. In those submissions, Mr. Farago indicated in part:

(...)

As I indicated in my letter of July 15, 2016 (Case Record pages 17 to 19), cost relief was originally denied in a letter dated July 28, 2015 on the basis that there was "no evidence to support a pre-existing condition caused or contributed to the injury, enhanced the injury, or delayed recovery".

I indicated that [the worker] sustained a significant physical injury on March 25, 2015, including a compression fracture at the T3 level of the spine. However, this was not accurately diagnosed until a CT scan was performed on April 21, 2015 (Case Record page 185). Following his injury, [the worker] required treatment that was not available at the remote job site where he had been working. The WSIB accepted that [the worker] should receive treatment near his home. [The worker] also attended the WSIB Back and Neck Specialty Clinic in Toronto on May 25, 2015.

The WSIB extended loss of earnings benefits to July 31, 2015 on the basis that the employer acknowledged that they did not have modified work available in [the worker's home city]. [The worker] was expected to return to his regular duties on July 31, 2015 (see decision dated July 14, 2015 at page 291-292 of the Case Record).

The reason [the worker] was paid benefits until July 31, 2015 was not due to non-cooperation but was due to the fact that he needed treatment and that modified work was not available at the remote job site.

(...)

It is our position that [the worker's] condition was not properly investigated by the WSIB and he was never provided the necessary treatment or the time to heal. That is the primary reason for the length of his recovery.

(...)

Although the WSIB has not paid benefits beyond July 31, 2015 and has not paid for [the worker's] treatment, this issue is still under appeal at the Appeals Services Division. There is a significant amount of new medical information that has been provided to the WSIB and this will have to be considered in determining ongoing entitlement.

(v) Reply submissions from the employer

[17] Mr. Lam provided reply submissions dated September 5, 2018 which have been reviewed and are included in Post-Hearing Addendum No. 2. In those submissions, Mr. Lam indicated in part:

The respondent letter continues to ignore the lengthy history of Anxieties and the Sledgehammer injury. The problems are well beyond the fall, but the ongoing issues are directly related. A medical statement from October 22, 2015 approx. the same time does not refute any of the extensive medical meetings and tests. It actually supports the worker was properly recovering, there is no reason for the individual not to return to work.

(...)

Hence, the latest medical further supports our claim the worker had physical injuries in the areas related to the Sledgehammer and was properly recovering. Instead of following the medical findings, made a personal decision to leave [the employer]. The appeal for SIEF has always been to recognize pre-existing conditions.

(...)

[The worker's] behaviour differed from the medical, although supports our contention of an unrelated condition. This individual suffered a prior disability that significantly and continues to enhance the latest injury. On this basis, we respectfully request greater entitlement to SIEF relief be applied to this injury.

(vi) Analysis

[18] Since the worker was injured in 2015, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[19] The Board's authority to establish the Second Injury and Enhancement Fund derives from section 98 of the WSIA, which states:

98 (1) The Board may establish a special reserve fund to meet losses that may arise from a disaster or other circumstance that, in the opinion of the Board, would unfairly burden the employers in any class.

[20] Pursuant to section 126 of the WSIA, the Tribunal is required to apply applicable Board policy. In this case, the Board has notified the Tribunal that one of the policies that apply to this appeal is *Operational Policy Manual* (OPM) Document No. 14-05-03, entitled "Second Injury and Enhancement Fund". This policy provides in part:

Policy

If a prior disability caused or contributed to the compensable accident, or if the period resulting from an accident becomes prolonged or enhanced due to a pre-existing

condition, all or part of the compensation and health care costs may be transferred from the accident employer in Schedule 1 to the SIEF.

Both physical and psychological disabilities are included.

Guidelines

There is no provision in the Act for the Fund to apply to Schedule II employers.

In situations where alcoholism plays a role in the causation of an accident, it is not considered to be a pre-existing condition with regard to the application of SIEF relief.

The objectives of this policy are to provide employers with financial relief when a pre-existing condition enhances or prolongs a work-related disability. It thereby encourages employers to hire workers with disabilities.

Definitions

Pre-accident disability is defined as a condition which has produced periods of disability in the past requiring treatment and disrupting employment.

Pre-existing condition is defined as an underlying or asymptomatic condition which only becomes manifest post-accident.

(...)

SIEF-application to employer costs

Medical significance of pre-existing condition*	Severity of accident**	Percentage of cost transfer***
Minor	Minor Moderate Major	50% 25% 0%
Moderate	Minor Moderate Major	75% 50% 25%
Major	Minor Moderate Major	90%-100% 75% 50%

NOTES

* The medical significance of a condition is assessed in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person. An associated pre-accident disability may not exist.

With psychological conditions, the possibility of prior psychic trauma resulting from life experience could be considered as evidence of vulnerability, and justify recommending relief to the employer, even in the absence of pre-existing psychological impairment.

** The severity of the accident is evaluated in terms of the accident history and approved definitions.

Accident History Components

- mechanics (lift, push, pull, fall, blow, etc.)
- position (kneeling, standing, sitting, squatting, bending, etc.)
- environment (lighting, temperature, weather conditions, terrain, etc.)

Definition – “Severity of Accident”

Minor: expected to cause non-disabling or minor disabling injury

Moderate: expected to cause disabling injury

Major: expected to cause serious disability probable permanent disability

*** The percentage of the total cost of the claim transferred to the SIEF.

[21] As can be seen from the table of relief found in the Board’s SIEF policy, the amount of SIEF relief granted in a particular claim is dependent on two variables: the severity of the accident and the medical significance of the pre-existing condition.

[22] With respect to the accident severity, Board policy requires that accidents be characterized as either “minor” (expected to cause non-disabling or minor disabling injury), “moderate” (expected to cause disabling injury) or “major” (expected to cause serious disability, probable permanent disability). In addition to these definitions, the policy suggests that the severity of the accident is to be evaluated in terms of the “accident history components,” which include “mechanics” (lift, push, pull, fall, blow, etc.), “position” (kneeling, standing, sitting, squatting, bending, etc.) and “environment” (lighting, temperature, weather conditions, terrain, etc.). In determining the severity of accident *Decision No. 1021/12* confirmed that, in dealing with this issue, one does not take into account what eventually transpired in a claim but rather, one reviews the mechanics of the accident and determines what outcome would reasonably be expected to occur.

[23] In this case, there does not appear to be any significant dispute about the mechanics about what happened on March 25, 2015. The worker’s comments in his Form 7 are consistent with the employer’s own Incident Investigation Report Form, which notes:

[The worker] was walking to the power house at the intake side to get to the roof to re-install roof panels. As the crew was walking to the ladder a few guys were slipping and sliding, some even fell. The guys gave everyone a heads up that it was very slippery. [The worker] slipped but managed to not fall, and then walked about five feet when he slipped and his feet went out from underneath him. He landed flat on his back and heard and felt his back crack at the location of his D-ring. J was on the ladder and asked if [the worker] was okay. [The worker] said he wasn’t sure. J went over to see if he was okay. Once a few other guys showed up, they helped [the worker] up and got him into the truck where he was brought to the [employer’s] site trailer.

[24] While neither of the representatives appears to have directly addressed the issue of the accident severity, I find I am in agreement with the Board’s operating level and the ARO, that the accident severity ought to have been characterized as “moderate”. This sudden and uncontrolled slip and fall onto the D-ring would, in my view, be reasonably expected to cause a disabling injury.

[25] With respect to the matter of the medical significance of any pre-existing condition, Board policy also requires that it be characterized as minor, moderate or major. The policy does not define these terms and indicates only that the medical significance of a pre-existing condition is assessed “in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person”. These provisions were interpreted as follows in *Decision No. 1582/07*:

I interpret the policy to mean that the medical significance of a pre-existing condition should be considered to be “minor” if it made the worker slightly more liable to develop a

disability of greater severity than a normal person, and that it should be considered “major” if it made the worker extremely liable to develop a disability of greater severity than a normal person. If the extent to which the pre-existing condition made the worker more liable to develop a disability of greater severity than a normal person was more than slight, but less than extreme, the medical significance of the pre-existing condition could be considered moderate.

[26] The primary position of the employer’s representative, as I understand it, is that there was a pre-existing psychological condition present in this case which prolonged or enhanced the worker’s injuries. As noted above, OPM Document No. 14-05-03 acknowledges that there may be situations where “prior psychic trauma resulting from life experience could be considered as evidence of vulnerability, and justify recommending relief to the employer, even in the absence of pre-existing psychological impairment”.

[27] While I accept that there may be appropriate cases when SIEF relief can be granted to an employer because a pre-existing psychiatric condition has enhanced or prolonged a worker’s recovery from his or her compensable injuries, I am not satisfied that this is one of those cases. The evidence in this case does not establish, on a balance of probabilities, that the worker had a pre-existing psychiatric condition. While the May 27, 2015 report from Dr. D. Hoffman (orthopaedics) indicates that “the patient does have a history of anxiety”, no further details are provided. In addition, while Dr. Rampersaud, in his report of May 25, 2015, advised that the worker “requires assessment and treatment with a pain psychologist or counsellor”, there is no suggestion that this recommendation was linked to any pre-existing psychological condition the worker may have had.

[28] The employer’s representative also appears to take the position that there may have been a pre-existing organic condition which enhanced or prolonged the worker’s recovery from his compensable injuries. In that regard, Mr. Lam referred to a “sledge hammer injury” which the worker sustained a number of years prior to the compensable accident. Dr. Rampersaud referred to this injury in his May 25, 2015 report, indicating that “he also reports that a sledge hammer hit his back approximately eight years ago, causing soft tissue damage”. Dr. Rampersaud also notes however that “he made a full recovery from this”. Dr. Rampersaud’s comments are consistent with the reporting provided by Dr. Hoffman. In his May 27, 2015 report, Dr. Hoffman noted that “he tells me that about seven years ago, he sustained an injury to his upper back, when he was hit by a hammer near the base of the neck. He was only off work for a day, however, and did not miss any additional time”. While I accept there is evidence that the worker sustained a prior injury to his back, the evidence does not establish, on a balance of probabilities, that the prior injury played any role of significance in enhancing or prolonging the worker’s recovery from the injuries he sustained in 2015.

[29] While the employer has submitted that pre-existing conditions were largely responsible for the extent and duration of the pain experienced by this worker, two orthopaedic surgeons have indicated otherwise. In his March 25, 2015 report, Dr. Rampersaud noted that “from the nature of his injury, as well as a concurrent compression fracture and a soft tissue injury, even without the concurrent fracture, his pain is certainly related to his injury”. Similarly, Dr. Hoffman, in his May 27, 2015 report, noted that “the presence of a T3 vertebral body fracture would explain the pain that he has been having. I anticipate that it will take about three months for this to fully resolve from the time of injury”.

[30] In reaching my conclusions in this appeal, I have also taken into account the following:

- In the Form 8 of March 26, 2015, the attending physician answered “no” to the question “Are you aware of any pre-existing or other conditions/factors that may impact recovery?”
- In a clinical note dated April 7, 2015, Dr. K. Agostino (family physician) noted that there was no history of previous injury and no “other red flags”.
- In an Acute Low Back Injuries Program of Care for Initial Assessment Report of April 10, 2015, Dr. S. Brimmell (chiropractor) answered “no” to the question “Are there any complicating factors that may delay recovery?”
- In a note dated May 2, 2015, Dr. Agostino noted “acute back pain from injury in March. CT indicates T3 compression fracture that correlates clinically”.
- In a Health Professional’s Progress Report dated July 21, 2015, Dr. Brimmell answered “no” to the question “Are you aware of any pre-existing or other conditions/factors that would impact return to work or recovery?”
- I was not referred to medical evidence of any significance, dealing specifically with the facts of this worker’s claim, that explained how pre-existing organic or psychological conditions enhanced or prolonged the worker’s recovery from the injuries he sustained in March, 2015.

[31] For these reasons, I find the employer has failed to establish, on a balance of probabilities, that there was a pre-existing condition which enhanced or prolonged the worker’s recovery. As such, the employer’s request for SIEF relief is denied.

DISPOSITION

[32] The employer's appeal is denied.

DATED: November 1, 2018

SIGNED: R. Nairn